

The COVID-19 Pandemic and the Push to Promote and Include Nurses in Public Health Policy

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The response to the COVID-19 pandemic highlighted the microcosm of public health and health equity challenges that we are experiencing in the United States. The pandemic response also heightened the importance of including nursing perspectives when addressing public health issues. However, nurses are largely excluded from national public policy conversations about the pandemic response. Here we explore the historical, structural, and social factors that created and perpetuated this dynamic and offer ways to amplify the visibility and influence of nursing perspectives in shaping public health policy at the academic, health system, and research levels.

NURSES: A CRITICAL WORKFORCE ESSENTIAL TO HEALTH CARE

Nurses represent a sizable workforce essential to health care and play a critical role in promoting and implementing

public health policy. Nursing is the largest health care and support profession in the United States, with more than 5.8 million US nurses overall (registered nurses, licensed practical or vocational nurses, and nursing assistants) and 3.8 million registered nurses (<https://bit.ly/35sWKSj>).¹ Nurses provide care in multisector community settings across the care continuum and have unique patient-adjacent perspectives and skills that can aid public health policy development, including interpersonal and communication skills and support and management skills (Figure 1).

In outpatient settings, nurses are educating communities, administering community-based and home-based specialty care, and serving as community care team coordinators. At the bedside, nurses are routinely leveraging their training and skills to transform access to and delivery of health care across communities and sectors through quality improvement initiatives, research, and policy advocacy. Nurses

are also critical in disaster response and mitigation efforts. The COVID-19 pandemic highlighted how nurses endure to provide compassionate and competent care when caring for critically ill and infectious patients, despite challenging work environments. This multifaceted expertise makes nurses ideally suited for cross-collaborative policy development roles (<https://bit.ly/3NEQVcO>).^{2,3} However, despite these invaluable positions and skills, nurses are notably underrepresented in public health media and policy discussions.

Broadening public understanding and awareness of the rigorous training, unique knowledge and skills, and essential roles of nurses is imperative to increasing nurse visibility and inclusion in policy, media, and decision-making spaces. For instance, nurses receive extensive health care training and skill development in disease treatment, management and prevention, and health and wellness promotion. Nurses excel at interdisciplinary team leadership and are experts in patient education, adapting treatments and crafting self-management regimens.

Nurses are also most often responsible for disease and public health monitoring, assessment, and planning decisions. In hospital and care settings, nurses often spearhead efforts to enhance care quality by leading quality improvement efforts, care delivery system planning and operations, and resource allocation. Nurses' unique set of knowledge, skills, expertise, and perspective is critical to informing and crafting local, state, and national public health innovation and transformation. Neglecting to include them in policy-making conversations and decisions is detrimental to patients, communities, and national health.

Entry-Level Nurse Competencies		Advanced-Practice Nurse Competencies
Interpersonal and Communication Skills		
 Conflict resolution, team building, and delegation	2.2d Demonstrate ability to conduct sensitive or difficult conversations 6.4c Engage in constructive communication to facilitate conflict management	6.4h Manage disagreements, conflicts, and challenging conversations among team members 6.4i Promote an environment that advances interprofessional learning
 Collaboration with patients, family, other health professional team members, and leaders from other disciplines	2.2a Demonstrate relationship-centered care. 6.3a Integrate the roles and responsibilities of health care professionals through interprofessional collaborative practice	2.2g Demonstrate advanced communication skills and techniques using a variety of modalities with diverse audiences 6.3d Direct interprofessional activities and initiatives.
 Advocacy for patients, communities, and the health professional team	3.4b Describe the impact on population outcomes, including social justice and health equity 3.5e Evaluate the effectiveness of advocacy actions	3.4g Design comprehensive advocacy strategies to support the policy process 3.5i Demonstrate leadership skills to promote advocacy efforts that include principles of social justice, diversity, equity, and inclusion
 Communication skills through active listening; expressing ideas and information clearly, verbally, and in writing; and emotional intelligence	2.2c Use a variety of communication modes appropriate for the context 2.2f Demonstrate emotional intelligence in communications	2.2h Design evidence-based, person-centered engagement materials
Support Skills		
 Compassionate attitude and striving to understand	2.1a Demonstrate qualities of empathy 2.1b Demonstrate compassionate care	2.1d Promote caring relationships to effect positive outcomes 2.1e Foster caring relationships
 Being willing to make a change and influencing others to set and achieve goals	2.7c Recognize the need for modifications to standard practice	2.7f Synthesize outcome data to inform evidence-based practice, guidelines, and policies
Management Skills		
 Strategic thinking, problem solving, and decision-making	3.3a Describe access and equity implications of proposed intervention(s) 7.1a Describe organizational structure, mission, vision, philosophy, and values	3.3c Analyze cost-benefits of selected population-based interventions 7.1e Participate in organizational strategic planning
 Clinical leadership for patient care practices and delivery, including the design, coordination, and evaluation of care for individuals, families, groups, and populations	3.3b Prioritize patient-focused and/or community action plans that are safe, effective, and efficient in the context of available resources	2.9i Analyze system-level and public policy influence on care coordination
 Accountability for evaluation and improvement of point-of-care outcomes, including the synthesis of data and other evidence to evaluate and achieve optimal outcomes	3.4c Identify best evidence to support policy development	3.4f Identify opportunities to influence the policy process
 Lateral integration of care for individuals and cohorts of patients	3.2a Engage with other health professionals to address population health issues	3.2e Challenge biases and barriers that impact population health outcomes
 Information management or the use of information systems and technologies to improve health care outcomes	8.1b Identify the basic concepts of electronic health, mobile health, and telehealth systems for enabling patient care 8.3e Identify impact of information and communication technology on quality and safety of care	8.1g Identify best evidence and practices 8.3k Pose strategies to reduce inequities in digital access to data and information.
 Stewardship and leveraging of human, environmental, and material resources	3.6a Identify changes in conditions that might indicate a disaster or public health emergency	3.6f Collaboratively initiate rapid response activities to protect population health
Knowledge and Expertise		
 Embracing professional development and life-long learning through education, reflective learning, courses, seminars, and programs	10.2a Engage in guided and spontaneous reflection of one's practice	10.2i Foster activities that support a culture of lifelong learning
 Credibility of competency by practicing ethically, and the ability to use ethical considerations to guide decisions and actions	9.1c Demonstrate ethical behaviors in practice	9.1h Analyze current policies and practices in the context of an ethical framework
 Managing care, knowledge of technology, patient safety, and resource management	8.2b Explain how data entered on one patient impacts public and population health data	8.2h Generate information and knowledge from health information technology databases
 Participation in identification and collection of care outcomes	2.5a Engage the individual and team in plan development	2.5j Develop evidence-based interventions to improve outcomes and safety
 Risk anticipation for individuals and cohorts of patients	2.5e Anticipate outcomes of care (expected, unexpected, and potentially adverse)	2.5i Prioritize risk mitigation strategies to prevent or reduce adverse outcomes
 Design, implementation, and integration of evidence-based practice(s)	4.1b Demonstrate application of different levels of evidence 4.2a Evaluate clinical practice to generate questions to improve nursing care	4.1i Engage in scholarship to advance health 4.1i Disseminate one's scholarship to diverse audience using a variety of approaches or modalities 4.2k Evaluate outcomes and impact of new practices based on the evidence

FIGURE 1— Summary of Nursing Leadership Practice Competencies

Source. Information was adapted from the American Nurses Association Leadership Competencies; the American Association of Colleges of Nursing (AACN) Competencies and Curricular Expectations for Clinical Nurse Leader Education and Practice (<https://bit.ly/3NEQVcO>); *The Future of Nursing: Leading Change, Advancing Health* (<https://bit.ly/3DqDrp>); and the National Academies of Sciences, Engineering, and Medicine's "Future of Nursing 2020–2030: Charting a Path to Achieve Health Equity" (<https://bit.ly/3BKa9k>). Icons are from <http://thenounproject.com>. Entry-level and advanced-practice nurse competencies are from AACN's *The Essentials: Core Competencies for Professional Nursing Education* (<https://bit.ly/3ivPpLS>). Numbers refer to the domains and the two levels of subcompetencies as indicated in *The Essentials*.

UNDERREPRESENTATION OF NURSES IN HEALTH POLICY DISCUSSIONS

For more than 19 consecutive years, nursing has consistently ranked as the most trusted profession in the United States (<https://bit.ly/3uGdlC5>). However, evidence suggests that when journalists and media outlets are looking to find influential voices representing US public health, they rarely turn to nurses. The George Washington University Center for Health, Policy, and Media Engagement released a 20-year update of the original 1997 Woodhull Study on Nursing and the Media.⁴ The findings revealed that nurses were quoted in 2% of public commentaries in newspapers, magazines, and health industry publications covering health-related issues and were mentioned in only 13% of the media sampled. When cited, nurses were quoted on topics related to the profession of nursing itself, as opposed to their perspectives on or expertise in public health, health policy, or the health care industry. By contrast, physicians were quoted 10 times more frequently (21%) than nurses.

Media representation is consequential to policy-making agendas and action. The results from the updated Woodhull Study reflect the underrecognized value of nursing and the implications of perpetuated bias and inequity within the media. Notably, in the second phase of the recent Woodhull replication, researchers asked journalists and communication, public relations, and media staff from health care organizations and university settings to comment on why nursing expertise was not solicited more often.⁵

The study team found that although nurses were considered useful in

enriching a health-related news story, journalists did not believe that nurses were credible voices of authority. Interviewees also cited difficulty finding experienced nurses to interview and stated that health care organizations and universities often do not nominate their nursing staff as authority figures. In addition, the results suggested systemic barriers to nursing inclusivity at the level of editorial processes and policies that prevent nurses from being sourced. Finally, the findings revealed that the nursing profession was less strategic than other health professions (e.g., medicine) in engaging public health journalists.⁵

Although representing a sizable health care workforce, nurses are also significantly underrepresented in leadership arenas, adding another element to their public policy invisibility. For instance, among the thousands of US hospital networks, only 33 hospital chief executive officers are nurses (<https://bit.ly/3qR4P20>). Moreover, despite there being four times as many registered nurses as physicians in the United States, currently three nurses are members of Congress (<https://bit.ly/36LeuAf>), as compared with 17 physicians (<https://bit.ly/3JZN08e>). These statistics illustrate that limited progress has been made since the 2010 Institute of Medicine report *The Future of Nursing: Leading Change, Advancing Health* highlighted the value and critical importance of nursing perspectives and presence in decision-making roles (<https://bit.ly/3DqDrgp>).

Additional sociocultural and structural factors based on gender, media preferences, and medical hierarchy contribute to the invisibility of nurses in the media and in public health and policy discourses. Most apparent is that nursing is still a predominantly women-dominated

field, with more than 87% of nursing assistants and vocational and registered nurses identifying as women (<https://bit.ly/3uHZkDO>). Even as nursing becomes increasingly gender diverse, it continues to be deemed “women’s work,” undervalued both figuratively and literally as just part of the hospital “room and board.”⁶ In this regard, nursing is no different from other female-dominated fields wherein women are less likely than men to be called on as expert sources (<https://bit.ly/3iOLtWS>; <https://bit.ly/3K185za>).⁷

The continued omission of nursing perspectives in the media may also be partly due to the disinterest of lay or academic outlets in highlighting nurses’ work. Deena Costa, the senior author of the article “Ignoring Nurses: Media Coverage during the COVID-19 Pandemic,”⁸ shared that the study team initially received rejections from several lay newspapers and academic journals before the group’s work was published in the *Annals of the American Thoracic Society*. When asked about the etiology of these continued rejections, Costa replied that “some [publishers] said we were trying to do too much, and others said it wasn’t impactful enough. One journal just said they weren’t interested in it. Period” (D. Costa, personal communication, 2021).

Furthermore, perspectives about the value of nurses have deep roots within the medical hierarchy. Historically, nurses have been viewed as subservient and submissive to the male-dominated field of medicine, a likely nascent factor of this power imbalance.⁹ This long-held perception of nursing as a low-skilled, physician-servant profession ignores nurses’ tremendous levels of skill and autonomy and continues to be perpetuated through media misrepresentation. For example, *Saving Lives: Why the Media’s*

Representation of Nursing Puts Us All at Risk examines the role of nursing's misrepresentation and underrepresentation in the media in perpetuating stereotypes, including in many popular medical television shows in which physician characters offer all necessary care and nurses possess little expertise or value and are portrayed as peripheral servants.¹⁰

All of these synergistic and interrelated factors contribute to nursing's current low-impact position in public discussions about health policy. The implications of this underrepresentation and misrepresentation of nurses in national public health dialogues and policy agenda setting are far-reaching and detrimental to public health progress. The COVID-19 pandemic has presented a stark example of this invisibility and provides a ripe opportunity to shift the current dynamic toward greater nurse inclusivity in public health policy dialogues.

COVID-19: AN EXEMPLAR OF NURSING'S INVISIBILITY

The COVID-19 pandemic revealed a dismal picture of nursing representation in public health and health policy discussions and decision making. Staff nurses were at the forefront of advocating for policies on safe working environments, and nurse scientists found innovative solutions to the pandemic's challenges, including research examining delivery of care for patients with COVID-19 and the differential effects of the virus across populations.¹¹ However, no nurses were included in the Trump administration's Coronavirus Task Force. In response to this omission, Dermenchyan and Choi opined that nurses bring essential

knowledge, skills, and experience to health policy decision making, citing that a nurse on the task force would be an opportunity to include valuable interdisciplinary perspectives (<https://bit.ly/3qOal5m>).

This trend of omission from policymaking tables was not limited to the Trump administration; Joe Biden's Transition COVID-19 Advisory Board—originally composed of commissioners, physicians, public health experts, and academics—also did not include nurses (<https://bit.ly/3uK0Mpd>; <https://bit.ly/3iUVrG0>). In a CNN opinion piece, Ghazal and Dorsen argued that nurses could provide critical expertise for the advisory board, offering an important perspective regarding hospital staffing issues and resource shortages in health care systems (<https://cnn.it/3iTn2HG>). In response, Jane Hopkins, a psychiatric nurse, was later added to the board (<https://bit.ly/3iU3P8U>).

Underrecognition of nurses' role is not limited to national policy conversations. For instance, throughout the pandemic school nurses have frequently been left out of critical discussions about reopening schools safely. Many education officials have neglected to offer school nurses a seat at the table when decisions about COVID-19 school safety are being made, and in many cases nurses' recommended protocols have been ignored (<https://wapo.st/3JXdiZ5>).

The COVID-19 pandemic continues to place a significant strain on nurses' health, well-being, and ability to properly perform their jobs, and minoritized nurses have been disproportionately affected.¹² Particularly devastating are studies showing that nurses of color account for more than half of the deaths among nurses caused by COVID-19, despite representing only 24.1% of the workforce (<https://bit.ly/3iSBkZb>).

Furthermore, during the pandemic onset, nurses cited a concerning trend of fear of repercussions and retaliation in response to their advocacy for proper protective equipment and other hospital policies. Many nurses were ambivalent about being regarded as "health care heroes" in the media, which they felt was an oversimplification implying that they were willing to sacrifice themselves for others (<https://bit.ly/3LuROTI>). To add, this performative appreciation of nurses was coupled with omission of their voices from media coverage and policy planning and development.

RECOMMENDATIONS TO INCREASE NURSE VISIBILITY

Across fields and levels of education, nurses possess a multitude of transferable skills for public health policy development. Thus, it is paramount that policy development at the system, local, and national levels incorporate nursing representation. Moreover, academic institutions serve as a key entry point for journalists and policymakers to engage nurse experts. To this end, we highlight recommendations and practical tools to increase nursing visibility and influence in public health policy at the individual, community, systems, and national levels.¹³

Nurse Educators and Academic Institutions

As outlined in the newly revised Core Competencies for Professional Nursing Education of the American Association of Colleges of Nursing, there are missed opportunities to empower individual nurses to engage in policy and advocacy work during their nursing

education. Notable examples include integrating transformative learning experiences during nursing and clinical education, such as:

- Embedding structural (political) competencies and health policy curricula within nursing education at the associate, bachelor, master, and doctoral levels. Structural competencies should outline how policies shape the health care work environment and affect health at the community level.^{14,15}
- Teaching nurses the value of media presence and visibility by incorporating training on the use of social and lay media as a personal and professional advocacy tool.¹⁶
- Establishing interdisciplinary collaboration opportunities between schools of medicine, pharmacy, social work, and public health. Interdisciplinary collaboration promotes exposure of nurses and their expertise and enhances understanding of different roles and scopes of practice across disciplines.¹⁷ Collaboration may also promote a better understanding of complementary roles of nursing, medicine, and other disciplines.

Community Health Organizations and Health Systems

Leaders at the community and health systems level should leverage nurses' existing leadership, communication, and advocacy skills. Examples of ways to further build on and disseminate nurses' transdisciplinary lens and expertise include:

- Explicitly allocating seats for nurses on various committees, boards, and

quality improvement projects. Serving on boards allows nurses to partner with other leaders to promote change and advance health.¹⁸

- Enhancing community perceptions and knowledge of nurses' wide-ranging roles by referring journalists and media outlets seeking to publish health care–related stories to nurse informants.

Nurse Scientists

Nurse scientists possess additional leadership and interdisciplinary expertise transferable to health policy development and agenda setting. Opportunities to leverage research-trained nurses include:

- Increasing the number of doctorally prepared nurses engaged in the health policy process, whether through participation in institutional decision making, engaging with policies and procedures, or taking action at the local or national government level.
- Preparing and presenting policy briefs, writing letters to editors and elected officials on public health–related issues, and educating the lay public through speaking engagements with local community-based and civic organizations. These are excellent ways to directly influence the policy-making process.
- Expanding pathways for nurses to conduct policy-relevant and action-oriented research through fellowship training opportunities and involvement on boards (e.g., the Robert Wood Johnson Clinician Scholars Program, the National Clinician Scholars Program, and the Nurses on Boards Coalition).

CONCLUSIONS

The COVID-19 pandemic has underscored how nurses' voices have been underrepresented in public health policy. Nurses occupy a critical mass of the health care and public health domains and possess unique and transferable skills and perspectives relative to public health policy. However, they are often excluded as authority figures and content experts in public health policy development and agenda setting. The cyclical nature of the social and historical factors contributing to this exclusion with respect to media presence and public health policy leadership perpetuates such underrepresentation. Increasing the visibility of nurses will require collective promotion of the profession as a source of credible and competent expertise in promoting the health of communities and populations across settings including the academic, community, health system, and policy arenas. **AJPH**

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